

FOREIGN MR BREAST

Status: Final result

PACS Images

(Link Unavailable) Show images for FOREIGN MR BREAST

Study Result

Narrative & Impression

ACCESSION#

DATE: 12:33 PM

EXAM: REVIEW OF SUBMITTED BREAST IMAGING:

BILATERAL MAMMOGRAM WITH TOMOSYNTHESIS August 24, 2025:
Technique: bilateral mammogram: tomosynthesis images were obtained.

LEFT MAMMOGRAM WITH TOMOSYNTHESIS September 14, 2025:
Technique: left mammogram: tomosynthesis images were obtained.

BILATERAL LIMITED BREAST ULTRASOUND September 17, 2025:
Technique: bilateral breast us: limited breast ultrasound was performed.

LEFT LIMITED BREAST ULTRASOUND October 9, 2025:
Technique: left breast us: limited breast ultrasound was performed.

BILATERAL BREAST MRI WITH AND WITHOUT IV CONTRAST October 21, 2025:
Technique: bilateral breast MR: multiplanar, multisequence MR images were obtained before and after intravenous administration of gadolinium-based contrast agent.

CLINICAL HISTORY: Recently diagnosed two sites of DCIS with possible microinvasion 1-2:00 axis left breast (confirmed by MSK pathology) presenting for consultation. Evaluate outside imaging.

COMPARISON(S): October 2, 2025, August 20, 2024, and prior examinations dating back to June 10, 2022.

FINDINGS:

MAMMOGRAM:

BREAST DENSITY: Heterogeneously dense, which may obscure small masses.

RIGHT BREAST:

- * No
- * Benign-appearing lymph nodes project over the axillary region.

LEFT BREAST:

- * Upper outer quadrant, posterior depth interval development of microcalcifications on screening mammogram August 24, 2025. Subsequent diagnostic mammogram September 14, 2025 demonstrates 3.7 cm segmental distributed pleomorphic calcifications on the spot magnification views. The patient guided biopsy 1-2:00 axis October 2, 2025 demonstrating appropriate positioning of needle with target calcifications demonstrated in the provided specimen radiographs. Postprocedure mammogram demonstrates placement of a top hat shaped marker at the anterior aspect and hourglass shaped marker at the posterior aspect of the calcifications. Residual calcifications also span inferiorly by 2 cm from the top hat shaped marker.
- * Inner quadrant, mid depth, 1.0 cm oval circumscribed mass (tomo CC 29, MLO 34), benign and stable compared to prior examinations dating back to June 10, 2022.
- * Benign-appearing lymph nodes project over the axillary region.

ULTRASOUND: Evaluation is limited as interpretation is based on static images.

RIGHT BREAST:

- * No

RIGHT AXILLA:

No

LEFT BREAST:

- * 9:00 axis 2 cm from nipple oval avascular hypoechoic mass measures 0.8 x 0.8 x 0.5 cm on diagnostic ultrasound September 17, 2025, benign and mammographically stable compared to prior examinations dating back to June 10, 2022.
- * 1:00-2:00 axis postbiopsy changes at site of stereotactic biopsy on diagnostic ultrasound October 9, 2025.
- * No

LEFT AXILLA:

No

MRI:

BREAST COMPOSITION: Heterogeneous fibroglandular tissue with mild parenchymal enhancement.

RIGHT BREAST:

- * No

LEFT BREAST:

- * 1-2:00 axis, posterior depth, focal mass enhancement measures 2.4 x 1.6 x 1.3 cm (series 10 image 67; series 13 image 108), with biopsy markers located at the anterior and posterior aspect denoting the sites of DCIS (pending MSK pathology review). Area of enhancement abuts the chest wall without evidence of chest wall invasion.
- * 3:00 axis, posterior depth, located 0.6 cm anteroinferior to the proven malignancy, additional focal nonmass enhancement measures 1.4 x 0.5 x 0.7 cm, concerning for additional disease (series 10 image 57; series 13 image 105).
- * 9:00 axis, mid depth, 0.9 cm oval mildly enhancing mass without significant enhancement on subtraction images, corresponding to benign mass seen mammographically since 2022.

OTHER:

- * No
- * Scattered T2 hyperintense hepatic lesions measuring up to 1.8 cm in the left hepatic lobe (series 3 image 4) without discrete enhancement, probable cysts or hemangiomas.

ADDITIONAL IMAGING RECOMMENDED:

1. Depending on the treatment plan, if breast conservation is pursued, MRI guided biopsy of posterior left breast 3:00 axis focal nonmass enhancement.
2. Preoperative left diagnostic mammogram to assess marker positioning and residual calcifications.
3. If there is concern for microinvasion, targeted left axillary ultrasound for preoperative planning.

IMPRESSION:

1. 1-2:00 axis left breast DCIS with hourglass and top hat marker at the anterior and posterior aspect of calcifications.
2. 3:00 axis left breast focal nonmass enhancement concerning for additional disease and corresponding to calcifications spanning inferior from the biopsied calcifications.
3. No

Electronically Signed By: NEETAL BHAVE MD on 5:19 PM

Result History

FOREIGN MR BREAST (Order on - Order Result History Report

Supplies

						Charge
Name						
No						

 Implants

8:04 AM		Marino, Marlene	
Soft Tissue	Status	Implanted on	Expiration Date
ALLODERM CONTOUR MED/PERF THIN - SN/A -	Implanted		
Breast			
IMPLANT BREAST SMOOTH TISS EXP 133S-FV-12	Implanted		
400CC - S27817462 -			

Supplies

			Charge
Name			
No			

Signed by

Signed	Time	Phone	Pager
Neetal Bhave, MD	12:10		

Exam Information

Status	Exam Begun	Exam Ended
Final	12:36	12:36

External Results Report

There is an external results report available.

Encounter

[View Encounter](#)

Total Sedation Time

FOREIGN MR BREAST

Order:

Status: Final result Next appt: at 03:40 PM in Breast Medicine (Joshua Drago, MD)

Test Result Released: Yes (seen)

Details

Reading Physician

Neetal Bhave, MD

Routine

Narrative & Impression

ACCESSION#(s): A000262373, A000269662, A000270855, A000269663, A000269760

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* No [REDACTED]

ADDITIONAL IMAGING RECOMMENDED:

- IMPRESSION:

- Electronically Signed By: NEETAL BHAVE MD on [REDACTED] 5:19 PM

Last Resulted: [REDACTED] 12:10

 Order Details View Encounter Lab and Collection Details Routing Result History

 Patient [REDACTED]

 Released

 Seen

Study Details

Open Study Details